Baseline Data Quality Assessment (bDQA) Executive Report

Cohort 3 Health Facilities (13 Facilities) | Baseline Period: Q4 2025

1. Background, Rationale & Core Objectives

Data Quality Assessments (DQAs) serve as the foundation for clinical governance, health system accountability, and strategic planning. Data integrity degrades as information flows from primary registers to monthly summary sheets and electronic national databases (KHIS/DHIS2). This baseline report establishes performance benchmarks across 13 Cohort 3 facilities prior to targeted M&E; technical assistance.

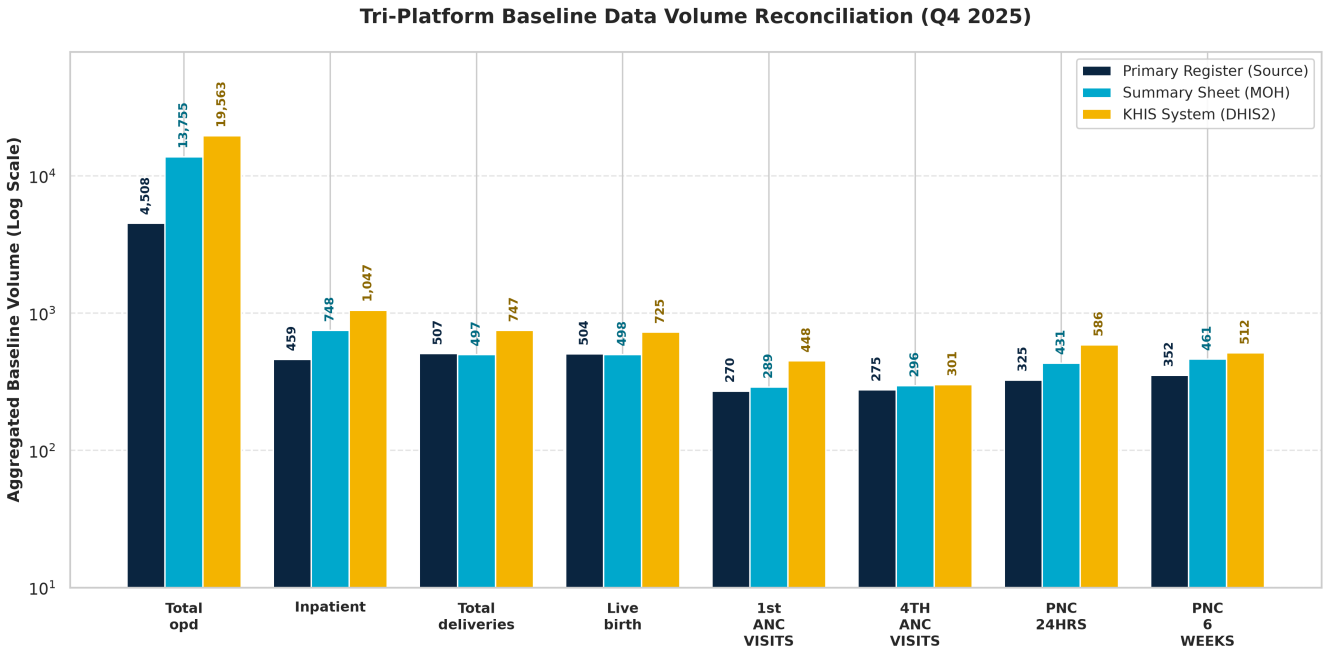
- **Tri-Platform Verification:** Reconcile clinical counts across Primary Registers, MOH Summary Sheets, and KHIS.
- **Clinical Logic Auditing:** Interrogate essential service cascades (Deliveries vs. Live Births/Stillbirths, Live Births vs. Immediate Breastfeeding).
- **Operational Vulnerability Mapping:** Identify root causes driving documentation gaps and transcription errors.
- **Actionable M&E; Roadmap:** Establish baseline performance tiers to guide targeted technical assistance.

2. Assessment Methodology Framework

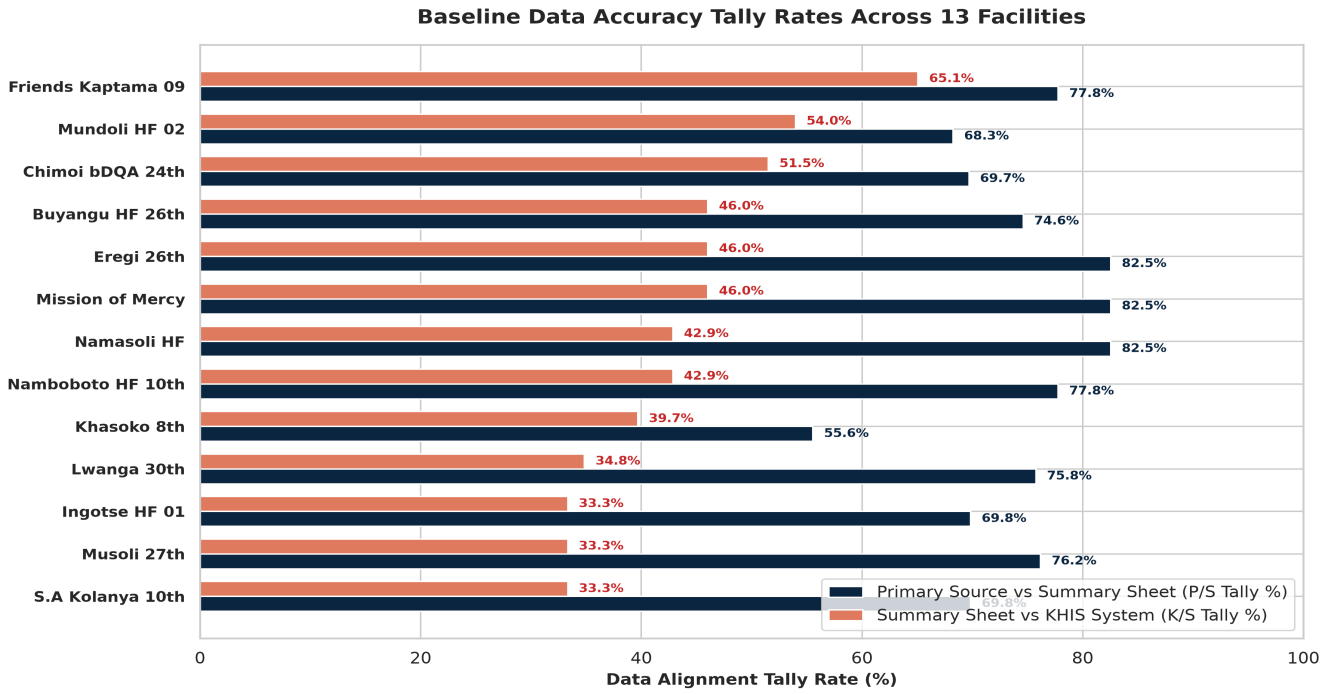
Reporting Level	Data Source Evaluated	Core Focus & Metric Evaluated
Tier 1: Primary Source	MOH Primary Registers (OPD, ANC, Delivery, PNC)	Direct patient encounter documentation at point of care.
Tier 2: Facility Tally	Monthly MOH Summary Sheets (MOH 705, 711, 717)	Manual aggregation of primary register counts at month-end.
Tier 3: National DHIS2	Kenya Health Information System (KHIS)	Electronic data uploaded to national DHIS2 repository.

3. Quantitative Findings & Clinical Consistency

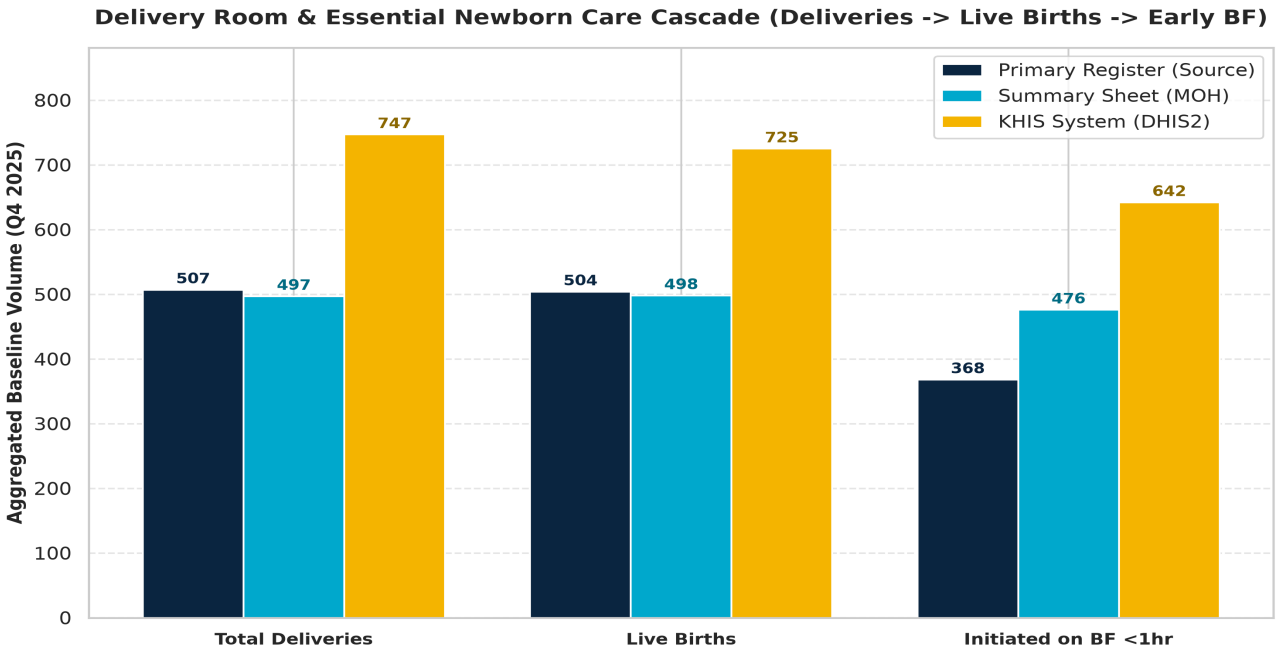
A. Platform Data Reconciliation Across Key Indicators



B. Facility Data Accuracy Performance Rankings



C. Delivery & Essential Newborn Care Clinical Cascade



4. Operational & Structural Root Causes

- **Improvised & Non-Standard Registers:** Shortages of official MOH primary registers forced facilities to use improvised notebooks lacking standard clinical columns.
- **Absence of Data Reviews:** No monthly data verification meetings (DRMs) occur at facility or sub-county levels prior to KHIS submission.
- **Minimal Supportive Supervision:** Irregular supervision from partners and SCHMT limits routine coaching opportunities.
- **Lack of Data Management SOPs:** Facilities lack formal SOPs for data tallies, archiving, and error validation.
- **Delegated Data Entry to Interns:** Sub-county HRIOs delegate KHIS uploads to casual clerks/interns without proper training or quality checks.
- **Completely Missing Datasets in KHIS:** Certain facility monthly reporting sheets were entirely missing from KHIS.

5. Strategic Recommendations & Action Plan

Identified Vulnerability	Targeted M&E; Intervention	Responsible Entity	Success Target
Improvised Registers	Procure and distribute standard MOH primary registers across all service areas.	LifeNet / SCHMT	100% register standardization
No Data Reviews	Establish mandatory monthly pre-submission Data Review Meetings (DRMs).	Facility In-Charges / LifeNet	Monthly DRM sign-off sheets
Erroneous Data Entry	Conduct targeted data entry & DHIS2 orientation for SCHMT interns and clerks.	Sub-County HRIO / LifeNet	Zero entry error rate
Lack of Governance	Develop, print, and distribute standardized Data Management SOPs.	LifeNet M&E Team	SOPs posted in all records units
Minimal Supervision	Institute bi-monthly joint supportive supervision visits (SCHMT + LifeNet).	SCHMT / LifeNet	100% supervision coverage